

It's Not Your Fault

A Practical Guide for Families Caring for an Aging Parent

Prepared for Dean Ferrera, MD · Rob Brizzi, CDP

Dr. Ferrera — rather than ask you to carry all 228 pages, here are five chapters that sit closest to your world, each shown exactly as a family meets it in the book: a 30-second box (the rule, the myth, the red line, and the words to use with you), the closing takeaway, and — where the book pauses to breathe — a one-page reflection. You told me this book "tends to the spirit." These five are where the logistics and the spirit meet.

"Plenty of guides cover the logistics. This one also tends to the spirit — the grief, the grace, and the promise at the center of caring for a parent."

— DEAN FERRERA, MD, CARDIOLOGIST

Something Has Changed

Where every family starts: the first change no one is sure how to name.

THIS CHAPTER IN 30 SECONDS

- **The rule:** patterns deserve attention — and a sudden change should always be taken seriously. The better first question is not "Is this normal aging?" — it is: what changed, when, and is it affecting safety?
- **The myth:** families think they need to be certain before they act. Caregiving does not require perfect certainty. It requires honest attention.
- **First move:** write down the change with facts and dates ("got lost driving to church on March 3") — vague worry is easy for a busy doctor to dismiss; concrete observations get taken seriously.
- **The script:** raise it with your parent first, and gently — "I've noticed the stairs seem harder lately — can we figure out something together?"
- **Watch for:** sudden change — confusion is sometimes caused by infection, dehydration, medication changes, pain, poor sleep, or low oxygen, problems that may be treatable. If the change is sudden, call the doctor; if the person may be in immediate danger, call 911.

The takeaway. You do not need to diagnose the problem today. The next right question is: what changed, and is anyone unsafe today?

CARDINAL MOMENT

A daughter once told me her mother was "completely fine" — and then, almost as an afterthought, that she had started keeping the mail in the refrigerator. She said it the way you would mention the weather. She had been explaining away small things for months, one at a time, because each one alone was easy to explain.

It was only when she said them out loud, all in a row, that she heard it. She cried — not because anything new had happened, but because she had finally let herself see what she already knew. Then she wiped her face and said, "Okay. What do I do first?"

That question is the beginning of everything. Noticing is not disloyalty. It is the first act of love for someone who can no longer fully carry themselves.

Caregiver Burnout

The caregiver becomes a second patient you never charted.

THIS CHAPTER IN 30 SECONDS

- **The truth:** burnout is not weakness, selfishness, or a lack of love — it is a warning sign. When a caregiver burns out, everyone becomes less safe.
- **The myth:** "I should be able to do this." Love does not remove exhaustion. Asking for help is not failure — it is planning.
- **The check:** take the burnout self-check, then ask the two most important questions: Is the caregiver safe? Is the person receiving care safe under the current plan? If either answer is no, the plan needs to change.
- **This week:** write down the actual workload, then ask by task — not "I need help," but "Can you take Mom to her appointment Thursday at 10?" Specific requests are harder to ignore.
- **If you are at the edge:** call or text 988 any hour, and ask the doctor, a social worker, or the Eldercare Locator (1-800-677-1116) about respite options today.

The takeaway. Caregiver burnout is not weakness. It is information. The next right question is: is the care plan protecting both the parent and the caregiver?

CARDINAL MOMENT

A son had slept in a recliner outside his mother's bedroom door for eight months, so he would hear her if she got up in the night. He never decided to become the night shift. It happened once, and then it simply never stopped.

He told me he was fine. His hands were shaking while he said it. We arranged two overnight shifts a week of paid help, and the morning after the first one, he slept until ten and woke up crying — partly from relief, partly from grief that he had waited so long to let anyone in.

Needing rest had never made him a worse son. Believing he was not allowed to have it had nearly made him a sick one.

Hospice

The benefit families reach for weeks too late, if at all.

THIS CHAPTER IN 30 SECONDS

- **What it is:** comfort-focused care for people who meet eligibility requirements and are nearing the end of life — a full team (nurse, aide, social worker, chaplain, volunteers, medical director) wherever the person lives. It is not giving up.
- **The trade-off:** while on hospice, Medicare generally stops paying for treatment intended to cure the terminal illness — care for other health conditions continues, comfort treatment continues, and the person can leave hospice at any time and return to regular Medicare coverage.
- **What it is not:** usually not a full-time bedside caregiver — the family, and sometimes private duty caregivers, still provide much of the hands-on daily care, so plan that coverage early.
- **Cost and the clock:** under the Medicare Hospice Benefit families usually pay almost nothing (often just a small copay of up to \$5 per comfort prescription); the roughly six-month prognosis is renewable with recertification, though room and board in a facility continues separately.
- **How to ask:** "Given the changes we are seeing, would a hospice evaluation be appropriate?" — an evaluation commits you to nothing, and the family always chooses.

The takeaway. Hospice is not the family choosing death. It is the family choosing support when the illness is already changing life. The next right question is: would comfort-focused care help us care better now?

Medical Orders — DNR, MOST, and POLST

Turning wishes into an order EMS can act on — and why a DNR is not "do not treat."

THIS CHAPTER IN 30 SECONDS

- **What it does:** a living will states wishes; a signed medical order — an out-of-hospital DNR, or a portable order form such as a MOST or POLST — gives instructions that care teams, including EMS, can act on; paramedics generally cannot follow a living will sitting in a folder.
- **What it is not:** a DNR is not "do not treat" — it addresses one thing, CPR; comfort, pain relief, dignity, and medical care all continue, and the order can be changed or voided at any time.
- **Watch for:** this chapter's worksheet is not a DNR, MOST, or POLST and has no legal or medical effect — only an order signed by a healthcare provider on your state's official form directs medical care.
- **How to ask:** "Which portable medical order form does our state use, and is it appropriate for my parent now?" — these forms are for people who are seriously ill or frail, not every healthy 70-year-old.
- **Where it lives:** keep the original visible — on the refrigerator, by the bed, or at the front of the chart — let it travel with the person, and review it at every care transition.

The takeaway. A DNR, MOST, or POLST turns a person's healthcare wishes into a medical order that care teams — including EMS — can act on, but only if it is signed, current, and findable. The next right question is: if the heart stops tonight, is there a signed order that says what to do, and can the responders find it?

What to Expect in the Last Days

What the final days actually look like, so the family is prepared, not blindsided.

THIS CHAPTER IN 30 SECONDS

- **What families may see:** more sleeping, less eating and drinking, breathing changes, cool hands or feet, restlessness, visions, sometimes a rally — every person is different. Even then, hearing may remain: speak as if they can hear.
- **Food is love, but:** offer, do not force — the family is not starving the person; the illness is changing the body. Often mouth care becomes more helpful than forcing fluids.
- **Breathing changes:** often the most frightening part — noisy breathing does not always mean the person is suffering, but still call the care team and ask for guidance. If on hospice, call hospice.
- **Comfort medications:** use them only as directed — families should never be left guessing. Ask what each is for, when and how much to give, and when to call.
- **If unsure, ask:** "We are seeing changes and we are not sure what they mean. Can you help us understand what is expected, what may be discomfort, and what we should do next?"

The takeaway. The last days may bring changes in sleep, eating, drinking, breathing, skin, awareness, and comfort needs. The next right question is: what are we seeing, who should we call, and what comfort support is needed now?

If these five ring true, the full manuscript and a ten-page clinical digest of all twenty-nine chapters are a word away. And if you'd be willing to expand your line into a few sentences — or a short foreword — in your personal capacity, it would tell a frightened family that the guidance here is safe to trust. With gratitude, **Rob Brizzi**.